

1. Administrative & Study Identification

Full Study Title: A phase II, open label, randomized, parallel arm study of NIS793 (with and without spartalizumab) in combination with SOC chemotherapy gemcitabine/nab-paclitaxel, and gemcitabine/nab-paclitaxel alone in first-line metastatic pancreatic ductal adenocarcinoma (mPDAC) **Short Title/Acronym:** daNIS-1 **Protocol Number:** CNIS793B12201 **NCT Number:** NCT04390763 **Posting ID:** 63827394175507268 **Sponsor/Company Name:** Novartis / Novartis Pharma AG **Investigational Product:** NIS793 (anti-TGF β monoclonal antibody), Spartalizumab (PD-1 inhibitor), Gemcitabine (Trade names: Gemzar, Infugem), Nab-paclitaxel **Phase of Development:** Phase II **Trial Status:** Terminated (Study was terminated early) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and tolerability of NIS793 in combination with spartalizumab and standard of care (SOC) chemotherapy during the safety run-in period; To evaluate Progression-Free Survival (PFS) per investigator assessment associated with NIS793, with and without spartalizumab, in combination with SOC versus SOC alone in the randomized phase. **Secondary Objectives:** To assess adverse events (AEs), serious adverse events (SAEs), dose reductions, dose interruptions, and dose intensity in the randomized phase, as well as Pharmacokinetics (PK), immunogenicity (IG), Overall Survival (OS), and Objective Response Rate (ORR).

2.2 Background & Rationale Overall survival remains low for patients with metastatic pancreatic ductal adenocarcinoma (mPDAC) despite approved therapies. PDAC is characterized by an abundance of intratumoral fibrosis that has been associated with a state of immune exclusion, constituting a mechanical obstacle to chemotherapy and contributing to the lack of immunotherapy efficacy. TGF- β is a pivotal activator of cancer-associated fibroblasts, leading to fibrotic networks. Combining a TGF- β targeting agent (NIS793) with immunotherapy (spartalizumab) and standard-of-care chemotherapy aims to overcome these barriers and improve clinical outcomes in treatment-naïve mPDAC. *(Note: In August 2023, Novartis discontinued the clinical development of NIS793, and the study was terminated early).*

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (gemcitabine/nab-paclitaxel alone) **Blinding:** Open-label **Randomization:** 1:1:1 ratio (following a safety run-in period) to one of three treatment arms: * Arm 1: NIS793 with spartalizumab and gemcitabine/nab-paclitaxel * Arm 2: NIS793 with gemcitabine/nab-paclitaxel * Arm 3: gemcitabine/nab-paclitaxel alone

3.2 Planned Enrollment & Duration Target N\$: 164 participants **Number of Sites:** ~30 sites across 15 countries **Estimated Study Start:** 22-Oct-2020 **Estimated Primary Completion:** N/A (Trial terminated early)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Signed informed consent must be obtained prior to participation in the study. 2. Male or female ≥ 18 years of age at the time of informed consent. 3. Participants with histologically or cytologically confirmed treatment-naïve metastatic adenocarcinoma of the pancreas with measurable disease as per RECIST 1.1. 4. Participants must have a site of disease amenable to biopsy, and be candidate for tumor biopsy according to the treating institution's guidelines. Participants must be willing to undergo a tumor biopsy at screening and during therapy on the study. In the event a new biopsy cannot be safely performed at study entry, an archival sample (collected <6 months prior) may be substituted following documented discussion with Novartis. 5. ECOG performance status ≤ 1 .

4.2 Exclusion Criteria 1. Previous radiotherapy, surgery (with exception of placement of biliary stent, which is allowed), chemotherapy or any other investigational therapy for the treatment of metastatic pancreatic cancer. Participants having received previous chemotherapy in the adjuvant setting. 2. Participants amenable to potentially curative resection. 3. Participants with a diagnosis of pancreatic neuroendocrine tumors (NETs), acinar, or islet cell tumors. 4. Having out of range laboratory values as pre-defined in the protocol. 5. Participants with MSI-H pancreatic adenocarcinoma. 6. Presence of symptomatic CNS metastases, or CNS metastases that require local CNS directed therapy (such as radiotherapy or surgery), or increasing doses of corticosteroids 2 weeks prior to study entry. 7. History of severe hypersensitivity reactions to any ingredient of study drug(s) and other mAbs and/or their excipients. 8. The participant exhibits any of the events outlined in the contraindications or special warnings and precautions sections of gemcitabine and nab-paclitaxel as per locally approved labels. 9. Impaired cardiac function or clinically significant cardiac disease. 10. Known history of testing positive HIV infection. 11.

Interruptions of NIS793, Spartalizumab, Gemcitabine and Nab-paclitaxel. 3. Randomized Part: Dose Intensity of NIS973 and Spartalizumab.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via onsite visits and laboratory tests; specific monitoring for immune-related AEs. AE grades based on CTCAE version 5.0 (Grade 1 = mild; Grade 2 = moderate; Grade 3 = severe; Grade 4 = life-threatening; Grade 5 = death related to AE). **Serious Adverse Events (SAEs):** Continuously monitored during the on-treatment period (up to 30 days post last dose). **Data Monitoring Committee (DMC):** Internal safety monitoring managed by Novartis Pharma AG.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes. Safety Set for AE/SAE evaluations. **Sample Size Justification:** 164 patients, intended to be appropriately powered to detect meaningful shifts in safety and tolerability. **Handling of Missing Data:** Standard imputation methods utilizing Last Observation Carried Forward (LOCF) for ongoing assessments prior to study discontinuation.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring. **Audit Trail:** Novartis Quality Management System (QMS) ensuring compliance with written protocols, ClinicalTrials.gov, and EudraCT reporting mandates.

11. Study Identifiers & Governance

Primary ID: CNIS793B12201 **Registry Number:** NCT04390763 **Posting ID:** 63827394175507268 **Sponsor/Lead Organization:** Novartis **Study Title:** daNIS-1 / Study of Efficacy and Safety of NIS793 (With and Without Spartalizumab) in Combination With SOC Chemotherapy in First-line Metastatic Pancreatic Ductal Adenocarcinoma (mPDAC) **Official Regulatory Title:** A Phase II, Open Label, Randomized, Parallel Arm Study of NIS793 (With and Without Spartalizumab) in Combination With SOC Chemotherapy Gemcitabine/Nab-paclitaxel, and Gemcitabine/Nab-paclitaxel Alone in First-line Metastatic Pancreatic Ductal Adenocarcinoma (mPDAC)

12. Clinical Framework (mPDAC Specific)

Primary Condition: Metastatic Pancreatic Ductal Adenocarcinoma (mPDAC) **Preferred UMLS Name:** Metastatic Pancreatic Ductal Adenocarcinoma **MeSH Code:** D064731 **SNOMED ID:** 415694002 **Diagnosis Threshold:** Histologically or cytologically confirmed metastatic adenocarcinoma **Medical Methodology:** Standard of Care Chemotherapy (Gemcitabine + Nab-paclitaxel) + Anti-TGF- β +/- Anti-PD-1

13. Study Procedures & Methodology

Management Pathway: 28-day treatment cycles with stringent disease monitoring **Education Protocol (HCP):** Protocol training, AE management for immune-related toxicities **Education Protocol (Patient):** Informed consent, biopsy procedures, infusion schedules **Quality Audit Mechanism:** Independent/Centralized RECIST review tracking & stringent PK/IG sampling

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until death, withdrawal, or early trial termination **Onsite Visit Cadence:** Days 1, 8, and 15 of every 28-day cycle **Remote Monitoring Frequency:** N/A (Intravenous therapy requires onsite administration) **Checklist Review Interval:** Efficacy scans every 8 weeks for the first year, then every 12 weeks

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				